

COMBINED MEDICAL RECORDS - PATRICK DONAHUE

DOB: September 14, 1981

Date of Loss: June 6, 2025

PROVIDER 1: SANTEE CHIROPRACTIC CLINIC

Provider: Michael P. Kelly, D.C.

Address: 9255 Carlton Hills Blvd Suite 5, Santee, CA 92071

Initial Visit: June 10, 2025

Chief Complaint: Acute cervical and lumbar pain following rear-end MVA June 6, 2025.

Diagnoses:

847.0 Cervical sprain and strain

847.2 Lumbar sprain and strain

723.4 Cervical radiculitis left C6

724.4 Lumbar radiculitis bilateral L4-L5 and L5-S1

719.41 Cervical facet syndrome

Cervical ROM: Flexion 30 degrees, pain rated 8/10 with movement.

Lumbar ROM: Flexion 25 degrees. Straight-leg raise positive at 45 degrees bilaterally.

Palpation: Moderate spasm C4-C7 paraspinals. Severe spasm L4-S1 paraspinals.

Neurological: Diminished sensation left C6 dermatome. Mild weakness hip flexion L3-L4.

Treatment: 3x per week chiropractic manipulation myofascial release EMS cold pack.

Discharge from chiropractic October 3, 2025. Referred to pain management.

MMI not reached at discharge.

PROVIDER 2: MAX MRI RADIOLOGY

Address: 8875 Aero Drive Suite 120, San Diego, CA 92123

Date of Study: August 14, 2025

MRI LUMBAR SPINE without contrast:

L3-L4: Mild disc bulge no significant stenosis.

L4-L5: Broad-based disc protrusion 5.2 mm eccentric left contacting left L5 nerve root.

Moderate left-sided neural foraminal stenosis. Facet hypertrophy bilaterally.

L5-S1: Posterior disc extrusion 7.1 mm with inferior migration. Severe bilateral foraminal stenosis. Contact with both S1 nerve roots. High-intensity zone at posterior annulus consistent with annular tear.

Impression: L4-L5 left paracentral disc protrusion with nerve root contact.

L5-S1 disc extrusion with annular tear and bilateral S1 root compression.

Findings consistent with acute traumatic disc injury.

MRI THORACIC SPINE without contrast:

T4-T5: Small posterior disc protrusion no cord compression. Otherwise unremarkable.

PROVIDER 3: COASTAL PAIN AND SPINAL DIAGNOSTICS

Provider: Ankush Kumar Bansal, M.D., Pain Management and Interventional Spine

Address: 6386 Alvarado Court Suite 350, San Diego, CA 92120

Initial Consultation: September 5, 2025

Referred by: Dr. Michael P. Kelly, D.C.

Patient: 44-year-old RV technician, rear-end MVA June 6, 2025.

Complaints: bilateral low back pain radiating into both legs (left greater than right), cervical pain with left arm numbness, difficulty sleeping, inability to perform job duties.

Diagnoses ICD-10:

M54.42 Lumbago with left-sided sciatica

M54.32 Sciatica left side

M47.816 Spondylosis with radiculopathy lumbar region

M51.16 Intervertebral disc degeneration lumbar region

M54.12 Radiculopathy cervical region

G54.2 Cervical root disorders

Examination:

Motor: 4/5 hip flexion L3-L4, 4-/5 left great toe extension EHL.

Sensory: Diminished pinprick L5 dermatome bilaterally, S1 left greater than right.

Reflexes: Absent left Achilles reflex.

Straight-leg raise: Positive at 40 degrees left, 50 degrees right.

Treatment Plan:

Lumbar medial branch block L3 L4 L5 S1 bilateral scheduled November 2025.

Physical therapy core stabilization lumbar traction 3x per week 6 weeks.

Cervical MRI recommended.

Medications: Meloxicam 15mg daily Cyclobenzaprine 5mg PRN Gabapentin 300mg TID.

Follow-up April 3, 2026:

Persistent severe bilateral lumbar radiculopathy. Unable to return to full RV technician duties.

Pain 7/10 at rest 9/10 with bending or lifting.

Medial branch block provided temporary relief 4 weeks; repeat procedure recommended.

Future Medical Needs estimated 5-year period:

Repeat lumbar medial branch blocks 3-4 per year: \$2,800-\$3,500 per session.

Cervical MRI: \$1,800-\$2,200.

Ongoing physical therapy: \$6,000-\$8,000 annually.

Medications: \$600-\$900 annually.

Total estimated future medical reserve: \$50,000.00 minimum.

Maximum Medical Improvement MMI: Not reached as of April 3, 2026.